جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : تقي ناصر محمود محمد

المحافظة : القاهره **الحي / المدينة :** الساحل **العنوان :** 21 ش علي مرسي الساحل

رقم البطاقة : 29401140101722 **رقم التليفون :** 01156504490 **تاريخ تسجيل الإستمارة :** 17-10-2026

العمر(تاريخ الميلاد) : 14-01-1994 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع:(----------)

المقر بما فيه
التوقيع:(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج كيماوي تاكسان (دوسيتاكل) [14152]

قسم العلاج : الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: تقي ناصر محمود محمد
National ID: 29401140101722

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Docetaxel chemotherapy**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

جوستاف روسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

المبادرة المصرية صحة

08/30/2026

MDT FORM

Name:	على ناصر مصيرة محمد	**MRN:**	13434
Age:	33 Y	**ID:**	29401140101722
Menopausal Sta	PEREMENOPAUSAL	**Phone:**	
NRS SCORE:		**Marital Status:**	MARRIED
BMI:		**Family History:**	-VE
CO Morbidity:	M:FREE , S:CS	**Reason Of MDT:**	NEW

Clinical Data:

BX: RT. ILC G2

ER: 0/8 **PR:** 0/8 **HER2:** 0 **KI67:** 30

Sonomammography:
ACR D; RT. ILL DEFINED SOLID MASS AT 12 O'CLOCK 23*20MM 3 CM FROM NIPPLE ,ANOTHER SMALLER LESION ANTERIOR TO INDEX BY 8 MM MEASURING 10*5MM
ONE PATHOLOGICAL LN SEEN AT LEVEL 1,
CESM: ILL DEFINED , ENHANCING 12 O,CLOCK 2.7CM FROM NIPPLE (INDEX MASS 17*17MM, SINILAR LESION 4MM MEDIAL TO INDEX LESION 9*5MM
PATHOLOGICAL LEVEL 1 LNS

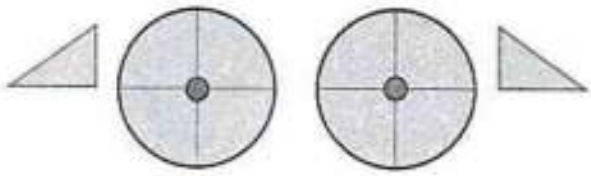
Mets Workup:
PET CT:RT. AXILLARY NODAL METS ,NO DISTANT METS

History:
33 YO FEMALE PT , FMH , WITH RT. ILC G2 TRIPLE NEGATIVE
PT. ADVISED TO REPLACE SC CONTRACEOTIVE CAPSULE WITH IUD
UPPER CENTRAL CLINICALLY T2 N1

Recommendations:
FOR POSSBBILITY OF CLIPPING BOTH LESIONS , NACT FOLLOWED BY RT. BCS +AXILLA ACCORDINGLY
2)BRCA TESTING
N:B PHOTO ATTACHED TO MDT GROUP
CESM HANDED TO FILMS

MDT Outcome & Plan
For:
1.BRCA testing
2.Clip insertion (in both lesions)
3.Portacath insertion then start NACT Taxol/Carbo weekly x12, followed by AC x4, with concomitant Keytruda 200mg q 3wks, guided by clinical response...then re-do mammosonography and proceed with RT BCS accordingly

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. شهاد الاشين	د. أمنية عيسوى				

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: تقى ناصر محمود محمد	
MR. : 13434	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 32-year-old individual with no history of diabetes mellitus or hypertension, no major surgeries, and a negative family history. She has three offspring, is on hormonal capsule therapy, and is perimenopausal. Two months ago she noted a right breast lump; imaging revealed a malignant solid lesion measuring 2.3 × 2 cm with pathological axillary lymph nodes. Subsequent contrast study identified two closely situated breast lesions with pathological axillary nodes. Core biopsy confirmed invasive ductal carcinoma, grade 2, triple-negative breast cancer (TNBC). PET-CT showed no distant metastases; the right upper outer quadrant harbored a neoplastic lesion 2.4 × 2 cm (SUV 17.8) with axillary nodes 2.3 × 1.5 cm (SUV 10). The case was discussed at multidisciplinary tumor board and a plan for neoadjuvant chemotherapy (NACT) combined with immunotherapy was recommended.

The patient attended today to initiate neoadjuvant chemotherapy. The regimen prescribed for week 1 includes paclitaxel (taxol) 80 mg/m², carboplatin at AUC 2, and pembrolizumab (Keytruda) starting at cycle 1. Laboratory values were reviewed and accepted; weight is 78 kg and height 172 cm. The patient was scheduled for subsequent pharmacy processing for week 2 and week 3. The patient has completed week 1 of taxol and cycle 1 of Keytruda.

The clinical pharmacy note indicates that Keytruda was unavailable at the time of dispensing. The pharmacy calculated a dose adjustment factor of 1.93 for taxol, resulting in a dose of 150 mg, and confirmed carboplatin at AUC 2. No further dosing cycles or weeks were specified. The current plan is to obtain Keytruda and continue the prescribed neoadjuvant regimen as outlined.

تحريرا في ,

التاريخ :	21-09-2026	**الوقت :**	

الطبيب المعالج : mohamed ismael

يعتمد ,

GUSTAVE ROUSSY

Patient Name	**نقى ناصر محمود محمد امام**	**Case No.**	**343630**
Sex/Age	**Female 32 Y**	**Patient No.**	**25099**
Registered Date	**22/7/2026 02:05:13 PM**	**Referred By Dr.**	**Out Patient**
Validation Date	**16/8/2026 12:21:22 PM**	**Branch**	**Gustave Roussy**
Reporting Date	**18/8/2026 10:25:00 AM**	**Pathology No.**	**2316/26**

Clinical Data: Right breast mass (BIRADS 4c).

Nature of Specimen: Tru-cut biopsy, collected on 22/07/2026, at 2:05 am. Cold ischemia time is 20 seconds.

Gross Examination:
-Four grey, white tissue cores, ranging in size between 0.8 cm & 1.0 cm long, totally submitted.

Technique:
- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:
-Histologic type: Invasive lobular carcinoma.
- Histologic grade: 2 (Differentiation 3, pleomorphism 2, mitosis 2)
- DCIS: Absent.
- LCIS: Absent.
- Angioinvasion: Absent.

Results:
* IPT (All controls showed appropriate reactivity):
- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Negative membrane reaction in tumor cells (score 0).
- Ki67%: 30%.

Conclusion:
Right breast mass, Tru-cut biopsy:
-Invasive lobular carcinoma, Grade 2.
IPT:
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).

[[DOCTYPE:BREAST_BIOPSY]]
Breast Biopsy

GUSTAVE ROUSSY INTERNATIONAL AGENCY

Gustave Roussy International Egypt Pathology Lab
جوستاف روسي انترناشونال - مصر معمل الباثولوجي

جوستاف روسي

Patient Name	نقى ناصر محمود محمد امام	**Case No.**	**343630**
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Reporting Date	**18/8/2026 10:25:00 AM**	**Pathology No.**	**2316/26**

- Ki67%: 30%.

***Scoring system: "ER, PR"*
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
*** Scoring system: "Her2"*
Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature:
Prof. Dr. Maha Guimei.

CAP

[[DOCTYPE:MAMMOGRAM]]
Mammogram

Pat. Name: Tokka Nasser Mahmoud	Pat. ID: 3618565
Procedure Name: Computed Digital Mammogram -Bo	Date: 13/08/2026

CONTRAST ENHANCED SPECTRAL MAMMOGRAPHIC EXAMINATION OF BOTH BREASTS:

CLINICAL SUMMARY : right breast mass, for assessment.
IMPRESSION :
LEFT BREAST: (BIRADS 1)

- Normal study

RIGHT BREAST: (BIRADS 5)

- 12 O'clock two closely related pathological mass enhancement with ipsilateral pathologically enlarged axillary lymph nodes.

DETAILED FINDINGS :
TECHNIQUE:
Craniocaudal & Mediolateral Oblique views
Contrast mammography/ ultrasound

Mild back ground parenchymal enhancement.

LEFT BREAST:

- LOW ENERGY (NON CONTRAST) IMAGES: No dominant mass lesion, spiculations or suspicious microcalcifications.
- RECOMBINED (CONTRAST IMAGES): showed no pathological uptake.
- No signs of malignancy detected.
- Normal skin/nipple/areolar complex.
- Preserved axillary lymph nodes.

RIGHT BREAST:

[[DOCTYPE:CT_SCAN]]
CT Scan

- LOW ENERGY (NON CONTRAST) IMAGES: ill defined spiculated mass at 12 o'clock.
- RECOMBINED (CONTRAST IMAGES): ill defined heterogeneously enhancing mass located at 12 o'clock 2.7 cm from the nipple (index mass) measure 17x17 mm.
- Similar smaller lesion located 4 mm medial to the index mass measure 9x5mm.
- Normal skin/nipple/areolar complex.
- Pathologically enlarged axillary lymph nodes (level I) the largest measure .22x14mm.
- No chest wall invasion.

Thanks for your Reference,
Dr Hanaa Abo El Magd

Signature Dr hanaa
18/03/20129:29 PM

Signature Dr hanaa
24/10/2023 10:38 م

Mrs.: Toaa NASER
Date : 20/07/26

Digitized Mammography and Complementary Ultrasonography for Both Breasts Revealed:

Clinical Summary:

32 years old female with Right breast palpable lump ,from one month

Findings:

- **right breast**
- Very dense mammary parenchyma; ACR d may obscuring small lesions
- Center of upper quadrants shows subtle asymmetry
- No suspicious calcifications
- US examination depicted ill defined heterogeneous solid lesion seen at 12 o'clock measuring 23x20mm away from nipple by 3cm and skin by 3mm with penetrating blood vessels . other small similar lesion seen anterior to main lesion by 8mm measuring 10xx5mm
- One enlarged pathological lymph nodes is seen at level I measuring 21x12mm with loss of fatty hilum
- No pathological lymph node at level II , infra clavicular or supra clavicular

left breast

- Very dense mammary parenchyma; ACR d may obscuring small lesions
- No ,asymmetrical density or distortion identified,
- ultrasonography examination shows mottling parenchyma appearance cyst or solid lesion
- Normal skin thickness and contour.
- Non specific axillary lymph nodes

Impression:

Right breast (BIRADS 4c) Malignant looking solid lesion , for biopsy ,
Left breast (BIRADS 2), adenosis

Much obliged
Dr. Marwa M.Hassan

[[DOCTYPE:PET_SCAN]]
PET Scan

Imaging report

كايرو سكان للأشعة والتحليل
CAIRO SCAN Radiology & LABS

Pat. Name: Tokka Nasser Mahmoud

Pat. ID: 3618565

Date: 08/08/2026

Prof. Momen Khalil MD | Prof. Dorria Salem MD | Prof. Ashraf Selim MD | Prof. Hamed El Showaby MD | Prof. Nabil Arafa MD | Prof. Hussein Alwy MD | Prof. Nadia Hassan MD | Prof. Abir Madkour MD | Prof. Adel Nassar MD | Prof. Nowier Abou El Maaty MD | Prof. Aida El Hoshy MD

Whole Body F-18 FDG PET/CT EXAMINATION:

Clinical History:

- *32 years old patient with newly discovered right breast suspicious mass lesion.*
- ***PET/CT requested for assessment.***

Technique:

- *For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed simultaneously and used for attenuation correction, anatomical localization and diagnosis.*
- *Hepatic Reference activity~3.4.*



Report

Head and Neck:

* *No hypermetabolic cervical lymphadenopathy could be detected.*
* *Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.*
* *The brain shows uniform tracer distribution with NO hypermetabolic focal lesions.*

Chest:

* ***Metabolically active FDG avid right breast UOQ ST neoplastic lesion , measuring about2.4x2.2 cm with SUVmax~17.8 for pathological correlation.***



[[DOCTYPE:PET_SCAN]]
PET Scan

Imaging report

كايرو سكان
للأشعة والتحاليل
CAIRO SCAN
Radiology & LABS

Pat. Name: Tokka Nasser Mahmoud

Pat. ID: 3618565

Date: 08/08/2026

* ***Metabolically active FDG avid right axillary lymph nodes , measuring about 2.3x1.5 cm with SUVmax~10 ;metastatic.***



* *No hypermetabolic left axillary or mediastinal lymphadenopathy ,few non FDG avid sub cm left axillary lymph nodes.*
* *No FDG avid lung nodules or masses.*
* *There are no active pleural abnormalities.*
* *Physiologic FDG uptake is noted in the heart.*

Abdomen and Pelvis:

* *No hypermetabolic abdominal or pelvic lymphadenopathy.*
* *Physiologic diffuse FDG uptake is seen over average sized liver with homogenous parenchymatous texture. No FDG avid focal lesions detected.*
* *Physiologic diffuse FDG uptake is seen over average size spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.*
* *The pancreas shows normal FDG avidity.*
* *There are no active adrenal nodules.*
* *Physiologic FDG excretion is seen in the kidneys and bladder*
* *Non FDG avid bilateral adnexal cysts likely functional.*

Musculoskeletal:

* *Normal FDG activity is seen over the axial skeleton.*
* *No focal active osteoblastic or lytic lesions noted.*

Prof. Hussein Abou El Mokarem MD | Prof. Abla El Ibrashy MD | Prof. Atef Medkour MD | Prof. Sohair Nassef MD | Prof. Hussein Allayla MD | Prof. Nadia Hassan MD | Prof. Hamdi El Shafeey MD | Prof. Nabil Abdo MD | Prof. Dalia Salem M.D | Prof. Ashraf Salem MD | Prof. Mohsen Khafr MD

[[DOCTYPE:PET_SCAN]]
PET Scan

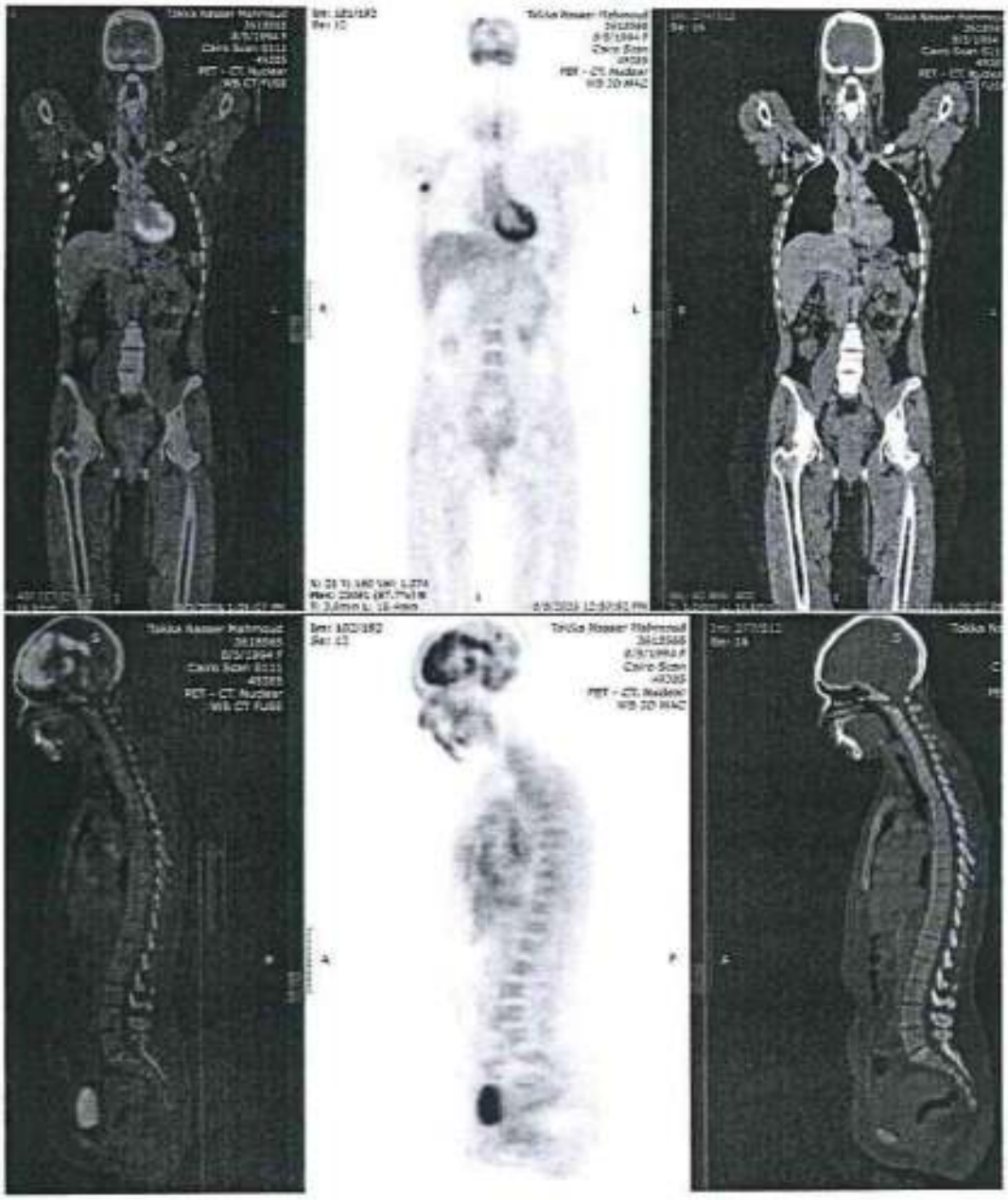
Imaging report

كايرو سكان
للأشعة والتحاليل
CAIRO SCAN
Radiology & LABS

Pat. Name: Tokka Nasser Mahmoud

Pat. ID: 3618565

Date: 08/08/2026



Prof. Hussein Abou El Makarem MD | Prof. Aida Maakbar MD | Prof. Hussein Afifi MD | Prof. Hamed El Showaby MD | Prof. Domia Salem MD | Prof. Mohsen Khalil MD
Prof. Aida El Idrissy MD | Prof. Sayed Nassef MD | Prof. Nahla Hassan MD | Prof. Nabil Attia MD | Prof. Ashraf Fahmi MD

[[DOCTYPE:PET_SCAN]]
PET Scan

Imaging report

كايرو سكان
للأشعة والتحليل
CAIRO SCAN
Radiology & LABS

Pat. Name: Tokka Nasser Mahmoud

Pat. ID: 3618565

Date: 08/08/2026

Conclusion:

PET-CT study showing:

- ***Metabolically active FDG avid right breast UOQ ST neoplastic lesion , with ipsilateral axillary nodal deposits for histopathological correlation.***
- ***No definite metabolically active FDG avid distant metastatic deposits detected.***

Thank you for referral

Dina Thabet , MSc
Nuclear medicine specialist
National Cancer Institute.

Ahmed Zaher , MD
Professor of nuclear medicine
National Cancer Institute

For any inquiries regarding the findings in the report please contact us on
radreport@cairoscan.com.eg

Prof. Hussein Abou El Mekarem MD | Prof. Aziz Mokkar MD | Prof. Hussein Afifyy MD | Prof. Komsa El Ghawaby MD | Prof. Doria Talem MD | Prof. Mohsen Khalil MD
Prof. Alaa El Ibrahiy MD | Prof. Sonia Nasser MD | Prof. Nagla Hosien MD | Prof. Hilbi Arida MD | Prof. Athar Bekhit MD

جوستافروسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنك المصرية هي صحة مصر

08/30/2026

MDT FORM

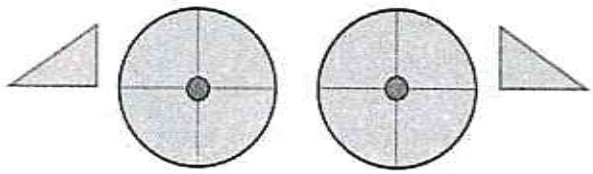
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Menopausal Sta	PEREMENOPAUSAL	**Phone:**	
NRS SCORE:		**Marital Status:**	MARRIED
BMI:		**Family History:**	-VE
CO Morbidity:	M:FREE , S:CS	**Reason Of MDT:**	NEW

Clinical Data:

BX: RT. ILC G2

ER: 0/8 **PR:** 0/8 **HER2:** 0 **KI67:** 30

R L



T N M

Sonomammography:

ACR D; RT. ILL DEFINED SOLID MASS AT 12 O'CLOCK 23*20MM 3 CM FROM NIPPLE ,ANOTHER SMALLER LESION ANTERIOR TO INDEX BY 8 MM MEASURING 10*5MM
ONE PATHOLOGICAL LN SEEN AT LEVEL 1,
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PATHOLOGICAL LEVEL 1 LNS

Mets Workup:

PET CT:RT. AXILLARY NODAL METS ,NO DISTANT METS

History:

33 YO FEMALE PT , FMH , WITH RT. ILC G2 TRIPLE NEGATIVE
PT. ADVISED TO REPLACE SC CONTRACEOTIVE CAPSULE WITH IUD
UPPER CENTRAL CLINICALLY T2 N1

Recommendations:

FOR POSSBBILITY OF CLIPPING BOTH LESIONS , NACT FOLLOWED BY RT. BCS +AXILLA ACCORDINGLY
2)BRCA TESTING
N:B PHOTO ATTACHED TO MDT GROUP
CESM HANDED TO FILMS

MDT Outcome & Plan

For:
1.BRCA testing
2.Clip insertion (in both lesions)
3.Portacath insertion then start NACT Taxol/Carbo weekly x12, followed by AC x4, with concomitant Keytruda 200mg q 3wks, guided by clinical response...then re-do mammosonography and proceed with RT BCS accordingly

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. شيماء لاشين	د. امنية عيسوي				

جوستاف روسي

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

البنك المصرية هي صحة مصر

08/30/2026

MDT FORM

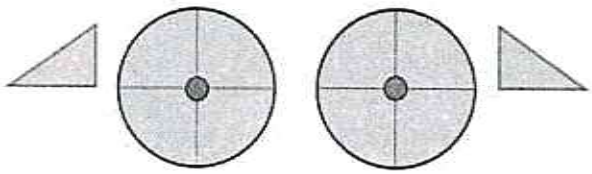
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Age:	33 Y	**ID:**	29401140101722
Menopausal Sta	PEREMENOPAUSAL	**Phone:**	
NRS SCORE:		**Marital Status:**	MARRIED
BMI:		**Family History:**	-VE
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R L



T N M

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SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. شيماء لاشين	د. امنية عيسوي				

National ID – Front & Back

[[DOCTYPE:ID_FRONT_BACK]]

جمهورية مصر العربية

بطاقة تحقيق الشخصية



تقى

ناصر محمود محمد امام

٢١ ش على مرسى

الساحل - القاهره

٢٩٤٠١١٤٠١٠١٧٢٢

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دبلوم سياحة وفنادق

أنثى مسلمة متزوجة

ايمن عطيه محمود نور

١ البطاقة سارية حتى ٢٠٣٢/٠١/٠٣